# Promotional HCP Email

*Fictional promotional email created for demonstration purposes. Length and content intentionally designed to illustrate editorial/QC review considerations.*

This sample demonstrates promotional content review, including identification of unsupported claims, risk assessment, and compliant revisions based on the presented clinical data.

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## Original Promotional Email (Provided for Review)

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**Subject Line:**
TELMORA™ Delivers Meaningful Clinical Responses in Patients With Post-JAK Myelofibrosis

Dear Dr. [Last Name],

Patients with myelofibrosis progressing after JAK inhibitor therapy have historically faced limited treatment options and poor outcomes. TELMORA™, a first-in-class telomerase inhibitor, demonstrated unprecedented clinical activity and durable responses in heavily pretreated patients with post-JAK myelofibrosis.

In the Phase II REVIVE-MF study, TELMORA achieved rapid and sustained spleen volume reduction alongside improvements in fatigue and overall quality of life. Many patients experienced clinically meaningful benefits regardless of prior treatment history, suggesting TELMORA may redefine expectations for this difficult-to-treat population.

Importantly, overall survival was not reached at the time of analysis, further supporting the long-term potential of TELMORA therapy. Investigators also observed encouraging hematologic improvements across multiple patient subgroups.

TELMORA was generally well tolerated, with adverse events manageable through dose modifications and supportive care. The safety profile remained consistent with previous studies and reinforced TELMORA's favourable benefit-risk profile in advanced disease.

As the first therapy to meaningfully address both disease burden and long-term outcomes after JAK inhibitor failure, TELMORA represents a promising advancement for patients with relapsed or refractory myelofibrosis.

Learn more about the evolving TELMORA data and clinical experience.

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## Editorial/QC Review Version

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**Subject Line:**
TELMORA™ Delivers ~~Meaningful~~ Clinical Responses in Patients With Post-JAK Myelofibrosis

Dear Dr. [Last Name],

Patients with myelofibrosis who progress~~ing~~ after JAK inhibitor therapy have ~~historically faced~~ limited treatment options and poor outcomes. In the Phase II REVIVE-MF study, TELMORA™, an investigational ~~first-in-class~~ telomerase inhibitor, ~~demonstrated unprecedented clinical activity and durable responses~~ was evaluated in heavily pretreated patients with post-JAK myelofibrosis.

**Commented [TL1]:** "Meaningful" may imply interpretation/promotion beyond endpoint data. Consider more specific efficacy language.

**Commented [TL2]:** "First-in-class" requires substantiation and may need regulatory approval context. "Unprecedented clinical activity" and "durable responses" are promotional overstatements unless directly supported by predefined endpoints and duration data.

~~In the Phase II REVIVE-MF study, TELMORA achieved rapid and sustained spleen volume reduction alongside improvements in fatigue and overall quality of life. Many patients experienced clinically meaningful benefits regardless of prior treatment history, suggesting TELMORA may redefine expectations for this difficult-to-treat population.~~

~~Importantly, overall survival was not reached at the time of analysis, further supporting the long-term potential of TELMORA therapy. Investigators also observed encouraging hematologic improvements across multiple patient subgroups.~~

TELMORA was ~~generally well tolerated,~~ associated with adverse events, which were manage~~dable~~ through dose modifications and supportive care. ~~The safety profile remained consistent with previous studies and reinforced TELMORA's favourable benefit-risk profile in advanced disease.~~

~~As the first therapy to meaningfully address both disease burden and long-term outcomes after JAK inhibitor failure, TELMORA represents a promising advancement for patients with relapsed or refractory myelofibrosis.~~

~~Learn more about the evolving TELMORA data and clinical experience.~~

**Commented [TL3]:** Provide supporting efficacy data for claims regarding spleen volume reduction and symptoms improvement. Consider including key endpoint results or revising claims to align with available evidence.

**Commented [TL4]:** Provide supporting data for survival and hematologic claims, including subgroup data where applicable. "OS not reached" should not imply survival benefit without mature supporting data.

**Commented [TL5]:** Provide quantitative data supporting safety and tolerability claims, including adverse events incidence and discontinuation data where applicable.

**Commented [TL6]:** Current positioning and long-term outcome claims require supporting clinical evidence. Remove unsupported efficacy and treatment-positioning conclusions.